

CLINICAL PHARMACOLOGY & MEDICAL INFORMATION REFERENCE MONOGRAPH

CARDIORIL (cardioril hydrochloride) TABLETS — DOSING IN RENAL IMPAIRMENT

Document Ref: MEDINFO-CARD-2025-019 | Target: Healthcare Professional Inquiries Only

1. CLINICAL DOSING RECOMMENDATIONS BY RENAL FUNCTION

Renal Function Category	eGFR (mL/min/1.73m²)	Initial Dose	Maximum Recommended Dose
Normal Renal Function	≥ 90	20 mg once daily	40 mg once daily
Mild Renal Impairment	60 – 89	20 mg once daily	40 mg once daily
Moderate Impairment (CKD 3)	30 – 59	10 mg once daily	20 mg once daily
Severe Impairment (CKD 4/5)	< 30 (non-dialysis)	5 mg once daily	10 mg once daily (titrate slowly)
End-Stage Renal Disease (ESRD)	Hemodialysis dependent	Contraindicated	Not Recommended

2. PHARMACOKINETICS & DIALYSIS CLEARANCE

Cardioril is 78% protein-bound. Renal clearance accounts for 42% of systemic drug elimination, with 58% cleared via fecal/biliary pathways. Due to high volume of distribution (Vd = 180 L), Cardioril is not significantly cleared by conventional 4-hour high-flux hemodialysis. Supplementary dosing post-dialysis is not required. Routine serum potassium monitoring is recommended in patients with baseline eGFR < 45 mL/min.

Regulatory Notice: This medical information document contains zero patient-specific clinical data, zero adverse drug reactions, and zero product quality complaints. It is classified exclusively under Medical Information (MI).